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Question 7

Not yet answered

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[Flag question](#)

A 20-year-old male patient presented with exertional dyspnea and palpitation of one year duration. Since one month, he started to have orthopnea, paroxysmal nocturnal dyspnea and head pounding. His blood pressure is 130/30mmHg, pulse rate is 120/min, respirations are 20/min. On Cardiovascular examination, arterial pulses are strong to-and-fro murmur audible at femoral artery and on precordial examination; there is a high-pitched, blowing, decre-scendo diastolic murmur, best heard in the third intercostal space. What is the most likely valvular lesion of this patient?

- ☐ a. Tricuspid regurgitation
- ☐ b. Mitral regurgitation
- ☐ c. Mitral valve prolapse
- ☐ d. Aortic regurgitation

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Question 2

Not yet answered

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 [Flag question](#)

A 10-minute-old baby was delivered from a 30-year-old P-III mother with an assisted vacuum delivery. The baby's fifth minute APGAR score was 5 and is currently on resuscitation with bag and face mask ventilation. The heart rate is 55/min. What is the most appropriate next step in the management of the newborn?

- ☐ a. Continue to resuscitate with bag and face mask
- ☐ b. Stop resuscitation and give the baby to parents
- ☐ c. Intubation and Epinephrine
- ☐ d. Chest compressions coordinated with PPV

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Question 8

Not yet answered

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[Flag question](#)

A 20-year-old married woman brought to a rape clinic by a police man with the complaint of rape of two hours duration. She gave history of anal assault by three assailants; one is lorry-driver and has multiple sexual partners. On physical examination, there was bruise at anal verge with non-foul smelling whitish discharge. What is the most appropriate preventive measure for this case?

- ☐ a. Post exposure prophylaxis
- ☐ b. STI prophylaxis
- ☐ c. Emergency contraception
- ☐ d. TAT

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MoE - Exit Exam

MoEEE

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Question 4

Not yet answered

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[Flag question](#)

A 16-year-old girl came to a gynecologic OPD for the complaint of failure to see menses. She also complained a progressively worsening lower abdominal pain since a year back which has an associated abdominal swelling. However, her breast development and general body habitués is growing in a similar fashion to her peers. On physical examination, she has female pattern pubic hair distribution and a 16-week sized abdominopelvic mass. Since she has not stated to have any kind of sexual contact, pelvic examination has not been done. What is the most likely cause of her problem?

- ☐ a. Müllerian Dysgenesis
- ☐ b. Longitudinal vaginal septum
- ☐ c. Imperforate hymen
- ☐ d. Ovarian tumor

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MoE - Exit Exam

MoEEE

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Question 6

Not yet answered

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[Flag question](#)

A 19-year-old college student, who had missed her menses for two months, came to a gynecology OPD for termination of pregnancy. She has a history of peptic ulcer disease. She has an ultrasound result at hand which showed single intrauterine gestational sac measuring 30mm, corresponding to an eight week pregnancy, but no fetal pole or yolk sac. What legal premise of Ethiopia allows her to get safe termination of pregnancy?

- ☐ a. Being a college student
- ☐ b. Her age
- ☐ c. The fetal status
- ☐ d. Her medical condition

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Question 5

Not yet answered

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 [Flag question](#)

A 17-year-old boy presented with high grade fever, nausea, vomiting of ingested matter and flank pain of three days duration. His temperature is 38.5°C, and pulse rate is 100/min. However, other physical examination findings are not remarkable. Laboratory result is notable for leukocytosis and pyuria. What is the most likely diagnosis of the boy?

- ☐ a. Acute cystitis
- ☐ b. Nephrolithiasis
- ☐ c. Acute prostatitis
- ☐ d. Acute pylenephritis

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Question 3

Not yet answered

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[Flag question](#)

A 16-hour-old male neonate was presented to a hospital with a complaint of failure to suck of five hours duration. He was delivered in a hospital at gestational age of 35-week. On examination, he has grunting, respirations were 68/min, pulse rate was 178/min, and has pale skin color. On laboratory investigations, hemoglobin level was 5g/dl, and blood group is A+ and O+ for him and his mother respectively. What is the most likely cause of the neonate's condition?

- ☐ a. Rh incompatibility
- ☐ b. ABO incompatibility
- ☐ c. Vitamin K deficiency
- ☐ d. Anemia of prematurity

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Question 61

Not yet answered

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 [Flag question](#)

A 54-year-old man came to a hospital with a complaint of vomiting of coffee ground matter of five hours duration. He has long standing epigastric pain, easy fatigability and dark tarry stool. At presentation, his blood pressure is 85/60mmHg and pulse rate is 120/min. His abdominal examination did not reveal remarkable finding. His laboratory findings show WBC count of $4.3 \times 10^3/\text{mm}^3$, platelet count of 178×10^3 and hemoglobin level of 7g/dl. What is the most appropriate initial management for the patient?

- ☐ a. Resuscitate and prepare for emergency laparotomy
- ☐ b. Secure IV line and send for upper GI endoscopy immediately
- ☐ c. Resuscitate with double IV line and prepare cross matched blood
- ☐ d. Put him on maintenance fluid and do NG tube lavage

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Question 68

Not yet answered

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 Flag question

A 60-year-old female known diabetic patient on follow-up was found to have hypertension. She is not symptomatic, but her blood pressure was in the hypertensive range for several months. She is taking ant-diabetic medications properly and her glycemic control is good. Currently, her blood pressure is 160/100 mm Hg. Laboratory investigation showed protein +2 in the urine otherwise normal. What is the preferred drug to start for blood pressure control?

- ☐ a. Amlodipine
- ☐ b. Hydrochlorothiazide
- ☐ c. Atenolol
- ☐ d. Enalapril

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Question 58

Not yet answered

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 [Flag question](#)

A 49-year-old man came with one day history of severe shortness of breath, cough and confusion. He is a known asthmatic patient with irregular follow-up and not adherent to medications. On examination, his blood pressure is 140/90 mmHg, respirations are 40/min and SPO₂ is 70percent at room air. The patient was admitted with emergency treatment protocol. What is the best initial treatment for this patient?

- ☐ a. Long acting beta agonists
- ☐ b. Systemic steroids
- ☐ c. Short acting beta agonists
- ☐ d. Magnesium sulphate

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Question 62

Not yet answered

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 [Flag question](#)

A 55-year-old diabetic patient was brought to a medical emergency OPD after sudden loss of consciousness while he was on bed two days ago. On evaluation; his temperature was 36.0 degree centigrade, pulse rate was 77/min, respirations were 23/min, and blood pressure was 160/100mmHg. On neurologic examination, the power on the left upper and lower extremity was 0/5 and GCS was 15/15. What is the most appropriate next step in the patient's care?

- ☐ a. Do brain CT
- ☐ b. Do brain MRI
- ☐ c. Start Atrovastatin
- ☐ d. Start Asprin

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Question 60

Not yet answered

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[Flag question](#)

A 52-year-old smoker was admitted and treated for heart failure secondary to ischemic heart disease. He improved significantly with no complaints. The patient's current vital signs are within the normal range. What is the best advice on discharge for long term benefit?

- ☐ a. Perform vigorous exercises.
- ☐ b. He can continue smoking as far as he continues his medication.
- ☐ c. He can start low dose alcohol for preventive effect.
- ☐ d. Stop smoking and continue his medication.

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Question 66

Not yet answered

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[Flag question](#)

A 55-year-old man came to an outpatient clinic with a complaint of intermittent epigastric burning pain which is worsened by taking food and awakes him from sleep for the last two months. He has no weight loss and any history of drug intake. Physical examination shows mild epigastric tenderness. Laboratory studies show hemoglobin level of 10.5g/dl, WBC count of 7500/mm³ and positive stool helicobacter antigen. Upper GI endoscopy shows duodenal ulcer. What is the best management for the patient?

- ☐ a. Start on H2 blocker
- ☐ b. Start on PPI +Clarithromycin +Metronidazole
- ☐ c. Start on proton pump inhibitor
- ☐ d. Start on H2 blocker +Clarithromycin +Metronidazole

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Question 59

Not yet answered

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[Flag question](#)

A 50-year-old female patient presents with a complaint of cough of two years duration. The cough is productive of small tenacious whitish sputum that is present in most days of the week. She also complains of occasional chest tightness, exertional dyspnea, fatigue and loss of appetite of same duration. She has no history of cigarette smoking. She uses wood for cooking. Physical examination shows scattered wheezing over chest fields. Chest x-ray shows hyper inflated lungs with flattening of diaphragm. What is the most likely diagnosis of this patient?

- ☐ a. Bronchiectasis
- ☐ b. COPD
- ☐ c. Obliterative bronchiolitis
- ☐ d. Bronchial asthma

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Question 65

Not yet answered

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 [Flag question](#)

A 55-year-old man came to an OPD with progressive left side chest pain. The pain starts while performing activities and improves during resting. He has smoked about 20 pack years. Physical examination showed normal system findings, but his blood pressure is 140/90 mmHg. What is the most likely cause of the patient's chest pain?

- ☐ a. Subacute pericarditis
- ☐ b. Atherosclerotic coronary artery disease
- ☐ c. Chronic pulmonary thromboembolism
- ☐ d. Obstructive valve disease

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Question 57

Not yet answered

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[Flag question](#)

A 48-year-old female patient was admitted to a medical ICU with the diagnosis of NYHA Class-IV Stage-C CHF secondary to chronic rheumatic valvular heart disease. The physicians and nurses in the ICU were unable to secure IV line. Where is the most appropriate site for IV access in the patient?

- ☐ a. Saphenous vein cut down
- ☐ b. Intraosseous route
- ☐ c. Scalp vein
- ☐ d. Jugular vein cut down

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Question 55

Not yet answered

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A 41-year-old man came to a hospital with a three weeks history of generalized body swelling that started from the legs and progressed to the whole body. He also has shortness of breath at rest and orthopnea of three pillows equivalent. On examination, his blood pressure is 130/80 mmHg, pulse rate is 120/min, respirations are 32/min, temperature is 35.8 degree centigrade, he has bilateral basal fine crepitation and S3 gallop, and TVLS is 14cm. Investigation showed Creatinine level of 2.1 mg/dl. What is the best initial treatment for the patient?

- ☐ a. Enalapril
- ☐ b. Metoprolol
- ☐ c. Hydrochlorothiazide
- ☐ d. Furosemide

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Question 69

Not yet answered

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 [Flag question](#)

A 60-year-old male patient needs to undergo urgent surgical intervention for small bowel obstruction. He was on aspirin for the last three years. What is the best component of blood that should be prepared in this patient care?

- ☐ a. Packed RBC
- ☐ b. Cryoprecipitate
- ☐ c. Whole blood
- ☐ d. Platelet

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Question 63

Not yet answered

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 [Flag question](#)

A 55-year-old female patient came to an emergency OPD with complaints of shortness of breath and right side chest pain of two days duration. Her temperature is 36.8 degree centigrade, respirations are 28/min, pulse rate is 104/min, and blood pressure is 80/50 mmHg. Cardio-respiratory examination shows that the lower posterior chest is dull to percussion and JVP is raised. CT angiography shows pulmonary artery thrombosis. What is the most effective management of this patient?

- ☐ a. IV fibrinolytic therapy
- ☐ b. Systemic glucocorticoids
- ☐ c. High dose Furosemide
- ☐ d. Oral anti-platelet

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Question 67

Not yet answered

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A 58-year-old man with abdominal distension and pain of two weeks duration visited a hospital. He also has blood mixed stool, tenesmus and weight loss. He was treated for intestinal parasitosis multiple times. On physical examination, his vital signs were within the normal range, abdomen was distended and hyper-tympanic, and bowel sounds were hyperactive. His hematocrite level was 32percent. Plain abdominal X-ray showed distended large bowel and barium enema showed apple core appearance. What is the most likely diagnosis of the patient?

- ☐ a. Colorectal Cancer
- ☐ b. Transverse Colon Volvulus
- ☐ c. Sigmoid Volvulus
- ☐ d. Cecal Volvulus

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Question 64

Not yet answered

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[Flag question](#)

A 55-year-old male patient presented to an emergency OPD with abdominal pain, distension, vomiting and failure to pass feces and flatus of three days duration. He had swelling on the left inguinal region for the last five years, which failed to reduce since four days. His physical examination results show a blood pressure of 100/60 mmHg, pulse rate of 112/min, temperature of 38.1 degree centigrade, abdomen is distended, and hypertympanic with decreased bowel sound, and there is a 12 cm x 15 cm non-reducible tender mass on the left inguinal region. What is the most likely diagnosis?

- ☐ a. Strangulated inguinal hernia
- ☐ b. Obstructed inguinal hernia
- ☐ c. Sliding inguinal hernia
- ☐ d. Incarcerated inguinal hernia

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Question 46

Not yet answered

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[Flag question](#)

A 36-year-old G-III, P-II woman comes to a regular OPD for her third ANC visit with a complaint of persistent head ache over the last one day. From LNMP, GA is 30 weeks. On physical examination, her blood pressure is 160/110 mmHg. She was admitted to the ward for further investigation and expectant management accordingly. What is the main aim of antihypertensive in this patient?

- ☐ a. To keep SBP between 130 and 140 mm Hg and DBP between 60 and 100 mm Hg
- ☐ b. To keep SBP between 90 and 120 mm Hg and DBP between 60 and 90 mm Hg
- ☐ c. To keep SBP between 120 and 140 mm Hg and DBP between 60 and 90 mm Hg
- ☐ d. To keep SBP between 140 and 155 mm Hg and DBP between 90 and 105 mm Hg

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Question 47

Not yet answered

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[Flag question](#)

A 36-year-old multi-parous woman has come to a gynecology OPD with a complaint of lower abdominal pain of one week duration which is associated with low grade fever and vaginal discharge. She has started using an IUD contraceptive since two weeks. On physical examination, she has cervical motion tenderness. What is the most likely diagnosis of the woman?

- ☐ a. Ovarian cyst torsion
- ☐ b. Uterine perforation
- ☐ c. Acute pyelonephritis
- ☐ d. Pelvic inflammatory disease

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Question 34

Not yet answered

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 [Flag question](#)

A 32-year-old Gravida IV Para III woman at her 34th week of gestation presented with complaints of abdominal pain and absent of fetal movement of 12 hours duration. Her prenatal course has been unremarkable. She denied vaginal bleeding or leakage of fluid per vagina. She has no history of trauma. All her previous deliveries were vaginally and uneventful. On physical examination, her blood pressure is 90/60 mmHg, pulse rate is 120/min, has pale conjunctiva, fundal height is 38 cm which is hypertonic with moderate tenderness, FHB is negative and cervix is closed. What is the most likely diagnosis?

- ☐ a. Uterine rupture
- ☐ b. Placenta previa
- ☐ c. Placental abruption
- ☐ d. Intra-amniotic infection

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Question 41

Not yet answered

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Flag question

A 35-year-old female patient came to a hospital with anterior neck swelling of five years duration. Since the last three month, she has started to experience palpitation and has a history of hot intolerance with no history of bone pain, cough or yellowish discoloration of the eye. On examination, her pulse rate is 110/min, blood pressure is 120/70mmHg, and there is lid lag and lid retraction. On thyroid examination, there is diffusely enlarged thyroid measuring of 4*3cm;the lower border is reachable. She is investigated with thyroid function test and the results show that TSH is 0.3micro unit/mL(Normal; 0.5–5 micro unit/mL), Total T4 is 180 nmol/L(Normal; 55–150 nmol/L), Total T3 is 5 nmol/L(Normal; 1.5–3.5 nmol/L). What is the most likely diagnosis of the patient?

- ☐ a. Toxic multinodular goiter
- ☐ b. Toxic Adenoma
- ☐ c. Graves' disease
- ☐ d. Follicular neoplasm

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Question 52

Not yet answered

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 [Flag question](#)

A 38-year-old female patient, who is on treatment for Crohn's disease for two years, came to a medical referral clinic complaining about weakness, palpitation, forgetfulness and headache of two months duration. On evaluation, she has tachycardia and absent deep tendon reflexes and position and vibration sensation. Her blood cell count shows WBC of 3.8×10^3 /microlitre, platelet of 350×10^3 /microlitre, and hemoglobin of 8.0g/dL. On examination of peripheral blood smear, there are three neutrophils with seven lobes. What is the most appropriate initial management for this patient?

- ☐ a. Prednisolone
- ☐ b. Cyanocobalamine
- ☐ c. Iron sulfate
- ☐ d. Immunotherapy

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Question 28

Not yet answered

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[Flag question](#)

A 29-year-old G-II, P-I mother, who is a known Graves's disease patient on treatment, presented to the regular OPD at her 36th weeks of gestation with complaints of palpitation, nervousness, sweating, and diarrhea of one day duration. On physical examination, she is anxious and confused, her blood pressure is 150/90 mmHg, pulse rate is 130/min, respirations are 28/min, temperature is 38.3°C and there is systolic ejection murmur at the apex. CBC results were; hematocrit level of 39percent, WBC count of $18 \times 10^3/\text{mm}^3$, and platelet count of $260 \times 10^3/\text{mm}^3$. What is the most likely diagnosis of this patient?

- ☐ a. Myxedema coma
- ☐ b. Thyroid storm
- ☐ c. Eclampsia
- ☐ d. Adrenal crisis

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Question 40

Not yet answered

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 Flag question

A 34-year-old primigravida lady at her 40th weeks of gestational age is admitted to a labor and delivery unit with a cervical dilation of 6cm, station 0, minimal caput and Grade-I molding. After four hours of evaluation, she is still at 6-cm cervical dilation with moderate caput, Grade-II molding and 0 station. What is the most likely diagnosis of the lady?

- ☐ a. Arrest of cervical dilation
- ☐ b. Protracted cervical dilation
- ☐ c. Protracted fetal descent
- ☐ d. Arrest of fetal descent

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Question 53

Not yet answered

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 [Flag question](#)

A 40-year-old female patient came to an OPD with the complaint of amenorrhea of three months duration. She also had frequent abortion for which she visited many private clinics and did not get cure. She also complained hot intolerance and significant weight loss. Her pulse rate is 120/min. On examination, she has lid lag and exophthalmia, diffused anterior neck swelling which moves with respiration, and thickened skin over the dorsum of the foot and pre-tibial region. What is the most likely explanation for these findings?

- ☐ a. Hashimoto's thyroiditis
- ☐ b. Secondary thyrotoxicosis
- ☐ c. Subacute thyroiditis
- ☐ d. Graves disease

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Question 38

Not yet answered

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[Flag question](#)

A 34-year-old male patient sustained stick injury to his arm. He had pain and is unable to use his extremity. On evaluation, his vital signs are in the normal range. There is visible deformity and crepitus over the left arm. Distal pulses are intact and there is wrist drop. What is the most likely explanation for this case?

- ☐ a. Humeral shaft fracture plus radial nerve injury
- ☐ b. Humeral shaft fracture plus radial and ulnar nerve injury
- ☐ c. Humeral shaft fracture plus median nerve injury
- ☐ d. Humeral shaft fracture plus ulnar nerve injury

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Question 48

Not yet answered

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 [Flag question](#)

A 36-year-old Para II woman came to a hospital with a complaint of failure to conceive of four years duration. She stated that her menses is irregular and do not have any feeling when it comes. She has no change in her appetite or body habitués. Her previous deliveries were attended vaginally with no complication. What is the most likely cause of infertility in this woman?

- ☐ a. Ovarian factor
- ☐ b. Cervical factor
- ☐ c. Uterine factor
- ☐ d. Tubal factor

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Question 45

Not yet answered

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[Flag question](#)

A 35-year-old woman presented with complaints of foul smelling greasy diarrhea, fatigability and weight loss. On examination, she is emaciated and has pale conjunctiva. Laboratory is notable for hemoglobin level of 8.5g/dl with MCV of 105fL. What is the most likely nutrient deficiency the patient has?

- ☐ a. Iron
- ☐ b. Cobalamin
- ☐ c. Copper
- ☐ d. Phosphorus

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Question 39

Not yet answered

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 [Flag question](#)

A 34-year-old pregnant woman at a gestational age of eight weeks has been presented to ANC for antenatal care. She has a history of pregnancy loss three times at the gestational age of 24 weeks, 22 weeks and 20 weeks. She had no pain during the expulsion process. She is worried about occurrence of similar condition in the current pregnancy. What is the most appropriate management for this patient?

- ☐ a. Bed rest
- ☐ b. Cerclage
- ☐ c. Progesterone
- ☐ d. Indomethacin

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Question 51

Not yet answered

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A 38-year-old daily laborer from Humera presents with complaints of fatigue, abdominal swelling and early satiety for the past six months. On physical examination, he is wasted, has pale conjunctiva and splenomegaly. His hemoglobin level is 9 g/dl, WBC count is 3000/mm³ and platelet count is 100000/mm³. His splenic aspirate microscopy revealed amastigote. PIHTC non-reactive (NR; Hemoglobin 13.3-16.2g/dl, WBC count 3.54–9.06 × 10³/mm³, platelet count 165–415 × 10³/mm³). What is the first line drug for the management of this patient?

- ☐ a. amphotercin B + paramomycin
- ☐ b. miltefosine+amphotercin B
- ☐ c. amphotercin B + sodiumstibulogluconate
- ☐ d. sodium stibulgluconate +paramomycin

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Question 36

Not yet answered

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 [Flag question](#)

A 33-year-old G-III, P-II woman, at 40 weeks' of gestation, presented with pushing down pain of four hours duration. She was admitted to labor and delivery unit and she was at 6cm of cervical dilatation. She was followed with partograph for labor progress. When she was re-evaluated after four hours, cervical dilation graph cross action line, with uterine contraction of 2/10/30-40", FHB is 140/min, membrane ruptured and clear liquor. What is the most appropriate next step in the management of this patient's care?

- ☐ a. Rehydrate her
- ☐ b. Augmentation
- ☐ c. Cesarean section
- ☐ d. Expectant management



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Question 56

Not yet answered

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[Flag question](#)

A 43-year-old Para I lady came to a hospital with a complaint of a menses which is small in amount and comes every 42 days. She does not have lower abdominal or back pain and breast tenderness during menses. What is the best explanation for this abnormality?

- ☐ a. Menopause
- ☐ b. Anovulation
- ☐ c. Adenomyosis
- ☐ d. Endometriosis

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Question 50

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 [Flag question](#)

A 37-year-old G-X, P-IX lady is going to have an emergency cesarean section for an indication of NRFHRP after she presents with repeated convulsions due to eclampsia. Currently, the convulsion as well as the blood pressure is controlled but she is still disoriented. She did not have an ANC follow-up. The husband has asked for a tubal ligation as they have decided not to have more children and they have no access to the service due to distance. What is the most appropriate measure for this condition?

- ☐ a. Appoint them for interval BTL
- ☐ b. Do tubal ligation during c-section
- ☐ c. Post-placental IUD insertion
- ☐ d. Give COC pills on discharge



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Question 42

Not yet answered

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Flag question

A 35-year-old female patient presents with colicky right upper quadrant abdominal pain of three months duration. Her pain worsens when she eats fatty meal. Since four days ago, she has persistent right upper quadrant pain, vomiting of ingested matter and fever. On physical examination, her blood pressure is 100/60mmHg, pulse rate is 112/min, temperature is 38.7 degree centigrade and she has tenderness during inspiration on right upper quadrant abdomen. On investigation WBC count was 12,000/mm³, and abdominal ultrasound shows gall bladder wall of 3.5mm thick and hyperechoic lesion with posterior shadow within it. What is the best initial management for this patient?

- ☐ a. Maintenance fluid, keep NPO and start sips when she tolerate
- ☐ b. Maintenance fluid, IV antibiotics, and urgent cholecystectomy
- ☐ c. Maintenance fluid, PO antibiotics and analgesics for seven days
- ☐ d. Maintenance fluid, IV antibiotics, analgesics and bowel rest

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Question 37

Not yet answered

Marked out of 1.00

 [Flag question](#)

A 33-year-old woman came to a clinic with complaints of decreased urine output and foamy urine of one week duration. On physical examination, a blood pressure of 150/90 mmHg, peri orbital edema and malar rash were noted. Investigation results show a urine analysis blood of +2, protein of +3 and many RBC cast, 24-hour urine protein of 2.5g and serum creatinine of 1.3mg/dl. What is the most likely cause of the patient's condition?

- ☐ a. Lupus nephritis
- ☐ b. Post streptococcus glomerulonephritis
- ☐ c. IgA nephropathy
- ☐ d. Focal segmental glomerulosclerosis

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Time left 2:54:20

Question 44

Not yet answered

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 [Flag question](#)

A 35-year-old male patient came to an emergency OPD complaining of high grade fever, headache and neck pain of three days duration. He has been admitted to a private hospital for three days taking unidentified medications but had no improvement. His temperature is 38.2 degree centigrade, pulse rate is 100/min and respirations are 24/min. On neurologic evaluation, he has neck stiffness and kerning sign is positive. Laboratory studies show; WBC count of $16.3 \times 10^3/\text{mm}^3$, platelet count of $480 \times 10^3/\text{mm}^3$, hemoglobin level of 14g/dl, there are 450 cells in CSF of which 75percent are neutrophils and he is positive for HIV. What is the most appropriate empiric management for the patient?

- ☐ a. Ceftazidime and Metronidazole
- ☐ b. Ceftriaxone, Prednisolone and Chloramphenicol
- ☐ c. Ceftriaxone, Vancomycin and Ampicillin
- ☐ d. Cefipime and Dexamethasone

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Question 32

Not yet answered

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[Flag question](#)

A 32-year-old G-II, P-I mother came for a routine antenatal care at 12th weeks of gestation. She has palpitation and fatigue. Her blood pressure is 110/70mmHg and pulse rate is 108/min. Laboratory findings are remarkable for hemoglobin level of 12.4g/dl, WBC count of $6 \times 10^3/\text{mm}^3$ and platelet count of $280 \times 10^3/\text{mm}^3$. What is the most likely explanation for these findings?

- ☐ a. Iron deficiency anemia
- ☐ b. Hypothyroidism
- ☐ c. Diabetes mellitus
- ☐ d. Hyperthyroidism

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Question 33

Not yet answered

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[Flag question](#)

A 32-year-old G-II, P-I woman was screened for diabetes at gestational age of 26 weeks with an indication of family history of diabetes and previous macrosomic infant. Her blood glucose level, one hour after ingestion of 50-g glucose load, was 145mg/dl. Then a 3-hour oral glucose challenge test was done and the results were as follows; FBS of 90mg/dl, 1-hour RBS of 160mg/dl, 2-hour RBS of 140 mg/dl and 3-hour RBS of 125 mg/dl. What is the most appropriate management option for this woman?

- ☐ a. Expectant management
- ☐ b. Metformin
- ☐ c. Glibenclamide
- ☐ d. Insulin

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Time left 2:55:34

Question 35

Not yet answered

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 [Flag question](#)

A 32-year-old male patient came to a hospital with the complaints of cough and severe left side chest pain while breathing for the last one week. His blood pressure is 110/60 mmHg, pulse rate is 100/min, respirations are 32/min and temperature is 38.4 degree centigrade. Respiratory system examination showed dullness and absent air entry over the left lower half of the lung fields. What is the most appropriate initial investigation for the patient?

- ☐ a. Echocardiography
- ☐ b. Chest x-ray
- ☐ c. Thoracentesis
- ☐ d. Complete blood count

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Question 54

Not yet answered

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[Flag question](#)

A 40-year-old woman came to an OPD with a complaint of post-coital bleeding. On pelvic examination, there was an irregular hard mass over the anterior lip of the cervix with estimated size of 3cm x 3cm. VIA was done and showed a deep acitowhite lesion which extends to the endocervix and the manipulation caused bleeding. What is the most appropriate next step in the management of this case?

- ☐ a. Punch biopsy
- ☐ b. Cryotherapy
- ☐ c. Hysterectomy
- ☐ d. LEEP

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Time left 2:53:33

Question 49

Not yet answered

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 [Flag question](#)

A 36-year-old woman who was operated for longstanding goiter presented to a surgical referral clinic with paresthesias of the distal extremities, numbness, painful muscle spasms, and seizures in the 3rd postoperative day. What is the most likely electrolyte abnormality in this case?

- ☐ a. Hypokalemia
- ☐ b. Hypocalcemia
- ☐ c. Hyponatremia
- ☐ d. Hypomagnesemia

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Time left 2:54:27

Question 43

Not yet answered

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[Flag question](#)

A 35-year-old Gravida II, Abortion I woman at the 8th month of amenorrhea came for antenatal care. She has no ANC visit before and has no any compliant. Her physical examination is non-revealing. Her laboratory result shows positive for HBsAg. She is counseled about the possibility of passing the infection to her baby and possible interventions. What is the most appropriate preventive measure to reduce risk of vertical transmission?

- ☐ a. HB vaccine series but avoid breast feeding
- ☐ b. Hepatitis B hyperimmune globulin only at birth
- ☐ c. Hepatitis B vaccine series and lamivudine
- ☐ d. HB immune globulin and HB vaccine series

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Question 31

Not yet answered

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[Flag question](#)

A 30-year-old G-II, P-I lady, who delivered her previous baby by cesarean section, came to an emergency OPD with a complaint of amenorrhea, vaginal spotting and mild back pain of two months duration. On physical examination, her vital signs were in the normal range, cervix is closed, uterus is about six-week sized, and cul-de-sac is not bulged and mass was not appreciated. Pelvic ultrasound showed empty uterus, no adnexal mass or free fluid seen. Her Serum HCG was 400IU. What is the most appropriate investigation in the diagnosis of the patient's case?

- ☐ a. Culdocentesis
- ☐ b. Serial hCG
- ☐ c. Serial HCT
- ☐ d. Laparotomy

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Question 30

Not yet answered

Marked out of 1.00

 [Flag question](#)

A 30-year-old female patient comes to a medical referral clinic complaining of bilateral recurrent pain and swelling of her hand joints for two years. She has no skin rash or photosensitivity. Her vital signs are in the normal range. Musculoskeletal examinations reveal swan neck like deformity of her hands. What is the most likely diagnosis of the patient?

- ☐ a. Osteoarthritis
- ☐ b. Reactive arthritis
- ☐ c. Gout
- ☐ d. Rheumatoid arthritis

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Question 29

Not yet answered

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[Flag question](#)

A 30-minute-old male neonate was presented with a complaint of fast breathing for 10 minutes. On physical examination, the neonate is in respiratory distress, respiration rate is 75/min, pulse rate is 154/min, temperature is 36.5 degree centigrade and gestational age is 43 weeks. Chest X-ray shows patchy infiltrates. What is the most likely diagnosis?

- ☐ a. Neonatal sepsis
- ☐ b. Birth asphyxia
- ☐ c. Meconium aspiration syndrome
- ☐ d. Hyaline membrane disease

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Question 27

Not yet answered

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[Flag question](#)

A 28-year-old male patient came to a health center after noticing skin discoloration on his back for unknown duration. Five hypo-pigmented skin lesions with loss of sensation on the lesion and enlarged ulnar nerves were found during examination. What is the most likely diagnosis?

- ☐ a. Leprosy
- ☐ b. Ulnar nerve neuropathy
- ☐ c. Tinea corporis
- ☐ d. Pityriasis alba

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Question 26

Not yet answered

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A 28-year-old Gravida II, Para I woman at her 38th week of gestation is admitted to a labour ward as active first stage of labor and on trial of labor after cesarean section. Her previous delivery was by cesarean section for mal-presentation. She has had adequate uterine contraction which has suddenly stopped and has developed minimal vagina bleeding after two hours of admission. Fetal heart rate was checked and became negative. What is the most likely cause for this condition?

- ☐ a. Placental abruption
- ☐ b. Uterine rupture
- ☐ c. Prolonged labor
- ☐ d. Vasa previa

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Question 47

Not yet answered

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[Flag question](#)

A 36-year-old multi-parous woman has come to a gynecology OPD with a complaint of lower abdominal pain of one week duration which is associated with low grade fever and vaginal discharge. She has started using an IUD contraceptive since two weeks. On physical examination, she has cervical motion tenderness. What is the most likely diagnosis of the woman?

- ☐ a. Ovarian cyst torsion
- ☐ b. Uterine perforation
- ☐ c. Acute pyelonephritis
- ☐ d. Pelvic inflammatory disease

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Question 24

Not yet answered

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[Flag question](#)

A 27-year-old patient came to an emergency unit complaining about nausea, vomiting, thirst, abdominal pain and shortness of breath for the last two days. On evaluation, temperature was 36.7 degree centigrade, respirations were 28/min, pulse rate was 116/min and blood pressure was 80/60mmHg. He is lethargic; however, he has no remarkable finding in other systems. His laboratory results reveal; WBC count of 4.8×10^3 /microlitre, platelet count of 156×10^3 /microlitre, hemoglobin level of 11.4g/dl, liver and kidney functions were normal, serum K^+ 4.5mmol/L (NR; 3.5-5.5mmol/L), serum Na^+ 140mmol/L (NR; 136-146mmol/L), RBS 560mg/dl and urine analysis shows ketone +3. What is the most appropriate initial step in the management of this patient?

- ☐ a. Metformin 1gm
- ☐ b. Start feeding by nasogastric tube
- ☐ c. 0.9percent saline 10 ml/kg/hr
- ☐ d. Ceftazidime 2gm

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Question 23

Not yet answered

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A 27-year-old male patient came to a medical OPD with a complaint of retrosternal chest pain of two days duration. On evaluation, his temperature is 36.5 degree centigrade, respiratory rate is 25/min, pulse rate is 92/min, and blood pressure is 100/60mmHg. There are no remarkable findings in other systems. Laboratory results show hemoglobin level of 12g/dl, WBC count of 5×10^3 /microlitre, and platelet count of 250,000/microlitre. Cardiac biomarkers are in the normal range. Chest X-ray findings are normal. ECG shows diffuse ST-segment elevations with upward concavity in leads I, II, AVF, and v2 to v6. What is the most likely diagnosis of this patient?

- ☐ a. Acute coronary syndrome
- ☐ b. Pericarditis
- ☐ c. Pulmonary thromboembolism
- ☐ d. Pneumonia

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Question 13

Not yet answered

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[Flag question](#)

A 24-year-old male patient came to an emergency OPD after he was injured at his back by an unknown offender three hours back. He had minimal bleeding from the stab site. On physical examination, he had 2 x 3cm deep laceration to his back, and the fascia and peritoneum were intact. What is the most appropriate management for the patient?

- ☐ a. Debridement, washing and primary closure
- ☐ b. Admit him, IV antibiotics and primary closure
- ☐ c. Wound care and delayed primary closure
- ☐ d. Wound care daily and let it close by itself

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Question 10

Not yet answered

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[Flag question](#)

A 21-year-old P-I mother was admitted with diagnosis of PID after she presented to an emergency OPD with the complaint of lower abdominal pain of five days duration. Associated with frequent vomiting and offensive vaginal discharge. On Physical examination, her temperature is 39.9 degree centigrade, has adnexal tenderness and cervical motion tenderness but no adnexal mass. What is the most likely indication for admission of this patient?

- ☐ a. Pelvic abscess is suspected
- ☐ b. Her age
- ☐ c. Diagnosis is uncertain
- ☐ d. Severity of the illness

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Question 11

Not yet answered

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A 22-year-old male patient presented with progressive mid-abdominal discomfort followed by anorexia, nausea and vomiting of 24 hours duration. The pain later shifted to the right iliac fossa. He had right lower quadrant abdominal tenderness and WBC count of $12000/\text{mm}^3$ with left shift (NR; $3.54\text{--}9.06 \times 10^3/\text{mm}^3$). What is the ultrasound finding that supports the most likely diagnosis?

- ☐ a. Presence of fluid in general peritoneum
- ☐ b. Length of appendix greater than 15mm
- ☐ c. Presence of air in general peritoneum
- ☐ d. Diameter of appendix greater 6mm

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Question 9

Not yet answered

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 [Flag question](#)

A 20-year-old primigravid lady at her 32nd week of gestation presented to an obstetrics triage complaining of right flank and back pain. She denies leakage of fluid or bleeding per vagina. Her antenatal history is unremarkable. On examination, her blood pressure is 100/60 mmHg, pulse rate is 104/min, temperature is 38.3°C, uterus is 32-week size, presentation is cephalic, uterine contractions are occurring every 3 to 5 minutes, and cervix is 3 cm dilated and 90percent effaced. Laboratory investigations reveal hemoglobin level is 13.5 g/dl, WBC count is $14.1 \times 10^3/\text{mm}^3$ and urine analysis shows nitrate +2. What is the most likely cause for the lady's condition?

- ☐ a. Pyelonephritis
- ☐ b. Placental abruption
- ☐ c. Intra-amniotic infection
- ☐ d. Cervical insufficiency

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Question 22

Not yet answered

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[Flag question](#)

A 26-year-old Para I woman came with a biopsy result which is suggestive of choriocarcinoma two months after evacuation is made for molar pregnancy. Further evaluation and metastatic work-up showed nothing except a slightly enlarged uterus and a bilateral adnexal cyst of 4 cm x 5 cm. Serum β -hCG is 25,000 mIU/mL (NR; 11,500-289,000 mIU/mL) and renal function test is normal. What is the most appropriate management for this woman?

- ☐ a. Etoposide
- ☐ b. Actinomycin D
- ☐ c. Dactinomycin
- ☐ d. Methotrexate

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Question 19

Not yet answered

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[Flag question](#)

A 25-year-old P-II mother came to a postnatal clinic a week after her delivery with complaints of fever and foul smelling discharge. On examination, her pulse rate is 108/min, temperature is 38.7 degree centigrade, and she has lower abdominal tenderness, cervical excitation tenderness and a purulent foul smelling vaginal discharge. What is the most likely cause of the patient's condition?

- ☐ a. Anaerobic bacteria
- ☐ b. Mixed infection
- ☐ c. Aerobic bacteria
- ☐ d. Gram negative bacteria

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Question 20

Not yet answered

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[Flag question](#)

A 25-year-old young man came to an emergency OPD after having dry cough, running nose fever and headache of three days duration. He did not have history of chronic lung problems and he is not smoker. His vital signs were within the normal range except temperature of 38.4 degree centigrade, he has clear chest and normal mental status. What is the most appropriate next step in the management of this patient?

- ☐ a. Do not give any medications
- ☐ b. Prescribe antibiotics
- ☐ c. Prescribe antiviral
- ☐ d. Prescribe antipyretics

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Question 21

Not yet answered

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Flag question

A 26-year-old male patient came to an OPD with complaint of pleuritic chest pain, shortness of breath, low grade fever and cough of two weeks duration. His blood pressure was 110/60mmHg, pulse rate was 86/min, respirations were 22/min and temperature was 37.9 degree centigrade. His Physical examination revealed presence of left lower chest dullness and absent air entry. Laboratory findings show WBC count of 12000/mm³, ESR of 55mm/hr and pleural fluid analysis shows WBC count of 1000 /mm³ with Lymphocyte of 70percent. What is the mostly likely diagnosis for this patient?

- ☐ a. Tuberculosis pleurisy
- ☐ b. Para pneumonic effusion
- ☐ c. Hepatic hydrothorax
- ☐ d. Malignant effusion

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Question 18

Not yet answered

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[Flag question](#)

A 25-year-old male patient came to a neurology clinic for his regular epilepsy follow-up. He has been taking the maximum dose of valproic acid, but he had six episodes of seizure in the last two months. He claimed that he is adherent to his medication. What is the most appropriate next step in the management of this patient?

- ☐ a. Taper Valproic acid and start regular doses of diazepam
- ☐ b. Add other anti-epileptic drug with different mechanism of action
- ☐ c. Add two drugs with similar mechanisms of action
- ☐ d. Discontinue Valproic acid and start other anti-epileptic drug

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Question 16

Not yet answered

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[Flag question](#)

A 25-year-old female patient came to a hospital with fatigue, shortness of breath and bilateral leg swelling of one month duration. She has no previously known chronic illness. Her blood pressure is 90/60 mmHg, pulse rate is 112/min, respiration rate is 18/min and temperature is 37.2 degree centigrade. She has right side pleural effusion, raised JVP, grade 2-3/6 holosystolic murmur at apex and bilateral leg swelling. Echocardiography showed left ventricular cavity diameter of 65 mm (normal value less than 54 mm) and left atrial diameter of 45 mm (normal value less than 38 mm) with ejection fraction of 30percent. What is the most likely underlying disease of the patient?

- ☐ a. Hypertrophic cardiomyopathy
- ☐ b. Myocarditis
- ☐ c. Dilated cardiomyopathy
- ☐ d. Restrictive cardiomyopathy

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Question 25

Not yet answered

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 [Flag question](#)

A 28-year-old G-II, P-I woman, who was ammenhorric for two months, came to a hospital to start ANC follow-up. On ultrasound examination, an eight week intrauterine pregnancy with negative FHB was seen. The result was disclosed to her and she preferred to go for medical method of termination. What is the most appropriate drug to be used for this woman?

- ☐ a. Oxytocine
- ☐ b. Methotrexate
- ☐ c. Misoprostole
- ☐ d. Hypertonoic saline

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Question 14

Not yet answered

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[Flag question](#)

A 24-year-old man came to a hospital with complaints of cough, fever and pleuritic chest pain of three days duration. He was treated with unspecified antibiotics for URTI two weeks back. On physical examination, his vital signs were unremarkable, but chest examination showed left middle lung field bronchial breath sound. What is the best initial management option for this patient?

- ☐ a. PO Augumentin plus Azithromycin
- ☐ b. PO Clarithromycin plus Doxycycline
- ☐ c. PO Doxycycline plus Ciprofloxacin
- ☐ d. PO Azithromycin

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Question 15

Not yet answered

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[Flag question](#)

A 24-year-old man has been on RHZE for the last three weeks for the diagnosis of pulmonary tuberculosis. In the last two week, the patient started to complain of burning and tingling sensation in his feet. What is the most appropriate step in the management of the patient's case?

- ☐ a. Reassurance and continue anti-TB
- ☐ b. Start on pyridoxine and continue anti-TB
- ☐ c. Discontinue only Isonazid and start on pyridoxine plus RZE
- ☐ d. Start on pyridoxine and stop all anti-TB

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Question 12

Not yet answered

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[Flag question](#)

A 23-year-old male patient who lives on the road side was admitted to a medical ward for acute febrile illness. He developed diaphoresis and palpitation hours after drug administration. On examination, his BP dropped to 80/40mmHg. What is the most appropriate next step in the management of this patient?

- ☐ a. Give IV fluid and dexamethasone
- ☐ b. Reassure the patient and continue the medication
- ☐ c. Send blood culture
- ☐ d. Transfuse with cross-matched blood

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Question 17

Not yet answered

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[Flag question](#)

A 25-year-old G-III, P-II Rh-negative woman presented to an ANC clinic at GA of 28 weeks with a positive indirect Coomb's test result. Her previous pregnancies were uneventful. Obstetric ultrasonographic examination shows no abnormalities. What is the most appropriate next step in the evaluation of the mother?

- ☐ a. Determine maternal titer
- ☐ b. Determine fetal antigen status
- ☐ c. Determine fetal HCT level
- ☐ d. Determine paternal zygosity

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Question 92

Not yet answered

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[Flag question](#)

A three-year-old male child was presented to a hospital with a complaint of generalized body swelling of five days duration. On examination, he had puffy face, and his blood pressure was between the 50th and 90th percentile. On urinalysis, +3 protein was revealed. Cholesterol level of 400 mg/dl, triglyceride level of 250 mg/dl, serum creatinine level of 0.4 mg/dl, and BUN level of 15 mg/dl were noted on laboratory investigations. What is the most appropriate management for the child?

- ☐ a. Start Amoxicillin 40 mg/kg/day
- ☐ b. Start Furosemide 4 mg/kg/day
- ☐ c. Start Enalapril 200 microgram/kg/day
- ☐ d. Start Prednisolone 60 mg/m²/day

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Question 73

Not yet answered

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 [Flag question](#)

A 75-year-old female patient came to a surgical OPD with complaints of difficulty of swallowing for three months. She also had hoarseness of voice. On evaluation, she was chronically sick looking. She had left supraclavicular LAP. What is the most appropriate initial diagnosis for this patient?

- ☐ a. Achalasia
- ☐ b. Lymphoma
- ☐ c. Esophageal cancer
- ☐ d. Tuberculous LAP

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Question 84

Not yet answered

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 [Flag question](#)

A physician was evaluating a CLD patient who is on propranolol and has had endoscopic band ligation one month back for primary prophylaxis of variceal bleeding. His blood pressure is 100/60mmHg and pulse rate is 92/min. On laboratory investigation, Hemoglobin level is 12.5g/dl and platelet count is 135,000/mm³. What clinical parameter is needed to optimize the propranolol dose?

- ☐ a. Platelet count
- ☐ b. Hemoglobin
- ☐ c. Pulse rate
- ☐ d. Blood pressure

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Question 85

Not yet answered

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 [Flag question](#)

A primipara mother has told a physician that she has difficulty of breast feeding her newborn baby. She is in the delivery room and getting ready for discharge. What is the most appropriate measure the physician should take?

- ☐ a. Evaluate her feeding technique to continue breast feeding
- ☐ b. Reassure her as most babies will have such problem and keep trying
- ☐ c. Evaluate her feeding to start formula milk until she feels comfortable
- ☐ d. Reassure her as the baby will suck better at home when she gets relaxed

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Question 77

Not yet answered

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A five-year-old male child is brought to a hospital with complaints of failure to use his left hip joint and pain of 10 days duration. Physical examination elicited a temperature of 38.9 degree centigrade, pulse rate of 112/min and slightly swollen right hip joint. He had also tender and restricted passive and active movements. The X-ray and ultrasound findings are normal. What is the most appropriate initial step in the management of this child?

- ☐ a. IV antibiotics and open joint biopsy
- ☐ b. IV antibiotics and follow change with ESR
- ☐ c. IV antibiotics and joint fluid gram stain and culture
- ☐ d. IV antibiotics and do CRP

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Question 100

Not yet answered

Marked out of 1.00

[Flag question](#)

An old diabetic patient is going to be operated for large bowel obstruction secondary to sigmoid volvulus. He was on oral glycemc agents for the last five years. No other medications. His vital sign is in the normal range. What is the best preoperative preparation for this patient?

- ☐ a. Making his blood sugar above 180mg/dl because of Intraoperative hypoglycemia
- ☐ b. Changing oral glycemc agents to regular insulin
- ☐ c. His blood sugar should be as low as possible due to risk of stress induced hyperglycemia
- ☐ d. He has to take morning dose of oral hypoglycemic agents with regular insuline

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Question 71

Not yet answered

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[Flag question](#)

A 62-year-old postmenopausal woman with a complaint of vaginal bleeding of three months duration came to a gynecologic OPD. Pelvic examination findings were; smooth cervix, no adnexal mass and a six-week sized uterus on bimanual examination. U/S evaluation showed two small intramural myomas measuring 2 x3cm each and endometrial thickness of 9mm. What is the most likely cause of the patient's condition?

- ☐ a. Myoma
- ☐ b. Cervical cancer
- ☐ c. Placental site trophoblastic tumor
- ☐ d. Endometrial hyperplasia

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Question 74

Not yet answered

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 [Flag question](#)

A female neonate, who was delivered at gestational age of 43 weeks, has been admitted to a NICU for the last five days after being referred due to respiratory distress immediately after delivery. Physical examination findings at admission revealed over-distended chest and Meconium stained extremities. Currently, she has severe hypoxia with clear chest and good air entry. Otherwise, she has no detected abnormality. The chest radiograph is normal. What is the most likely diagnosis for the current problem?

- ☐ a. Spontaneous tension Pneumothorax
- ☐ b. Bacterial superimposed infection
- ☐ c. Congenital diaphragmatic hernia
- ☐ d. Persistent pulmonary hypertension

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Question 80

Not yet answered

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[Flag question](#)

A mother has brought her three-month-old male infant to a clinic to get advice about his growth status. Anthropometry is done and all parameters are unaffected. What is the normal growth pattern to inform the woman?

- ☐ a. Head circumference increases by 2 cm per month for the next three months
- ☐ b. Weight increases by 30 g/day for the next three months
- ☐ c. Length increases by 0.5 cm per month for the next three months
- ☐ d. Weight doubles the birth weight at 4-5 months

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Question 76

Not yet answered

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Flag question

A five-year-old female child with complaints of fever and rash of three days duration is brought to a hospital. She also has cough, nasal discharge and diarrhea. On physical examination, her temperature is 37.8 degree centigrade, respirations are 43/min, pulse rate is 123/min, and there is diffuse red maculo-papular rash from face to leg. What complication will be expected in her third decade of life?

- ☐ a. Pneumonia
- ☐ b. Severe malnutrition
- ☐ c. Subacute sclerosing panencephalitis
- ☐ d. Measles encephalitis

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Question 81

Not yet answered

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 [Flag question](#)

A nine-hour-old male neonate was brought to a hospital with a complaint of failure to suck since birth. He was delivered at home and did not cry immediately after delivery. On physical examination, he has seizure on arrival; all neonatal reflexes are depressed, and lethargic. What is the first line treatment for this neonate?

- ☐ a. Phenytoin
- ☐ b. Phenobarbital
- ☐ c. Valproic acid
- ☐ d. Diazepam

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Question 79

Not yet answered

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 [Flag question](#)

A known diabetic patient for the last 15 years came to a regular OPD with the complaint of left calf pain of two months duration. The pain was aggravated during walking and relieved during rest. What is the most likely diagnosis of the patient?

- ☐ a. Diabetic neuropathy
- ☐ b. Deep vein thrombosis
- ☐ c. Varicose vein
- ☐ d. Peripheral arterial disease

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Question 72

Not yet answered

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[Flag question](#)

A 65-year-old, a known DM patient on metformin for last six years, presented with complaint of facial swelling for the last six weeks. He was diagnosed with Diabetic retinopathy before. Vital signs are in the normal range except his blood pressure is 150/95mmHg. He had periorbital and leg edema. On investigation, urinalysis albumin +4 blood +1, Cr-2.5mg/dl, 24 hour urine protein of 3.2g and HgA1c 8percent. What is the most appropriate management of this patient?

- ☐ a. Start on Furosemide
- ☐ b. Start on steroid
- ☐ c. Start on antibiotic
- ☐ d. Start on Enalapril

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Question 83

Not yet answered

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 [Flag question](#)

A patient who had been diagnosed with pulmonary tuberculosis came to an emergency room with a complaint of difficulty of breathing and chest pain. He is tachypenic and has signs of fluid collection on the right side which was tapped and found to be grossly purulent. His physician decided to insert chest tube. Where is the safest site of chest tube insertion on this patient?

- ☐ a. Fifth Intercostal space anterior to the mid-axillary line
- ☐ b. Fifth Intercostal space anterior to the anterior -axillary line
- ☐ c. Second Intercostal space below and lateral to the pectoralis major muscle
- ☐ d. Seventh Intercostal space below the level of the nipple

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Question 86

Not yet answered

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 [Flag question](#)

A seven-month-old infant is brought to a hospital for a routine follow up. Growth and development of the infant are optimal. A WBC count of 7,000/mm³, hemoglobin level of 6.7g/dl with MCV of 60fl, and RDW of 20percent were noted from laboratory work up. What is the most likely diagnosis of the child?

- ☐ a. Thalassemia
- ☐ b. Lead poisoning
- ☐ c. Iron-deficiency anemia
- ☐ d. Folate deficiency anemia

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Question 88

Not yet answered

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 [Flag question](#)

A six-week-old male infant was brought to a hospital with a complaint of rash for one hour. He took his first vaccine 12 hours back. On physical examination, his pulse rate is feeble, respirations are 65/min, he has stridor and wheezing over the chest, diffuse urticarial rash all over the body, and lip and oral mucosa swelling. What is the appropriate initial treatment for this infant?

- ☐ a. Blood transfusion
- ☐ b. Prednisolone
- ☐ c. Antihistamine
- ☐ d. Epinephrine

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Question 78

Not yet answered

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 [Flag question](#)

A four-year-old male child was brought by his mother to a clinic accompanied with a complaint of being unable to speak clear words. He clearly points to the thing he wants to have. He is able to ride tricycle and copies cross and circle on paper. Routine physical examinations did not show any abnormality. What is the most appropriate measure to be taken for the baby?

- ☐ a. Referring the child for mental retardation assessment
- ☐ b. Reassuring the mother to send him to regular school
- ☐ c. Reassuring the mother as it is normal
- ☐ d. Referring the child for hearing assessment

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Question 87

Not yet answered

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A seven-year-old known cardiac patient, with atrioventricular septal defect and who is on follow up, is planning to undergo dental correction with manipulation of gingival tissue. He has no known history of drug allergy. What is the best first-line endocarditis preventive strategy for this child?

- ☐ a. Cefepime
- ☐ b. Amoxicillin
- ☐ c. Azithromycin
- ☐ d. Clindamycin

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Question 89

Not yet answered

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[Flag question](#)

A three-year-old boy presented with inadequate intake and diarrhea of three days duration. His growth and development were optimal. On examination, skin pinch goes back very slowly and he is lethargic. His weight is 15kg. What is the most appropriate fluid therapy to this child?

- ☐ a. 1500ml RL over 24 hours
- ☐ b. 450ml RL over 30 minutes, followed by 1050ml over the next 2½ hours
- ☐ c. 450ml RL over 1 hour, followed by 1050ml over next 5 hours
- ☐ d. 300ml RL rapid bolus, followed by 450ml RL over 30minutes

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Question 82

Not yet answered

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 [Flag question](#)

A nurse working at a hospital sustained needle stick injury while carrying order for the patient with chronic liver disease with positive for hepatitis B surface antigen. The nurse had no history of liver disease and serology for hepatitis B is negative. What is the best management for this scenario?

- ☐ a. Do anti-HBs level and treat accordingly
- ☐ b. Reassurance and no treatment
- ☐ c. Give single dose of HBIG and single dose of HBV vaccine
- ☐ d. Give single dose of HBIG and full course of HBV vaccine

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Question 70

Not yet answered

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 [Flag question](#)

A 62-year-old man, who is a known HIV patient on AZT/3TC/EFV for the last two years with a good drug adherence, came to an adult OPD with complaints of headache and recurrent seizure of three weeks duration. Previously he was asymptomatic. His CD4 count was 400/microlitre six months back and currently it is 100/microlitre. What is the most likely explanation for his current problem?

- ☐ a. Acute stroke
- ☐ b. Treatment failure
- ☐ c. Drug side effects
- ☐ d. IRIS

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Time left 2:47:20

Question 90

Not yet answered

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[Flag question](#)

A three-year-old female child was presented to a hospital with a complaint of fear of light (photophobia) for three months. She also has skin rash more on the arm, leg and buttocks areas. On physical examination, she has foamy appearing plaques on conjunctiva and dry, scaly, hyperkeratotic patches over the leg, arm and buttocks. What is the most likely diagnosis?

- ☐ a. Thiamine deficiency
- ☐ b. Vitamin A deficiency
- ☐ c. Zinc deficiency
- ☐ d. Riboflavin deficiency

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Question 91

Not yet answered

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 [Flag question](#)

A three-year-old male child was presented to a hospital with a complaint of fever for five days. He also has a yellowish discoloration of the eye and frequent abnormal body movement of one day duration. He is from malaria endemic area. On physical examination, icteric sclera, splenomegaly and hepatomegaly were found. What is the best first line treatment for this child?

- ☐ a. Artesunate
- ☐ b. Coartum
- ☐ c. Quinine
- ☐ d. Arthemeter

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Time left 2:48:37

Question 82

Not yet answered

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 [Flag question](#)

A nurse working at a hospital sustained needle stick injury while carrying order for the patient with chronic liver disease with positive for hepatitis B surface antigen. The nurse had no history of liver disease and serology for hepatitis B is negative. What is the best management for this scenario?

- ☐ a. Do anti-HBs level and treat accordingly
- ☐ b. Reassurance and no treatment
- ☐ c. Give single dose of HBIG and single dose of HBV vaccine
- ☐ d. Give single dose of HBIG and full course of HBV vaccine

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Question 93

Not yet answered

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 [Flag question](#)

A two-hour-old male neonate was brought to a NICU after delivered from Hepatitis B surface antigen positive mother. On physical examination, his weight is 1500 gm; otherwise no other pertinent finding was detected. What is the most appropriate vaccination time of hepatitis B vaccine for this neonate?

- ☐ a. After weight reach 2.5kg
- ☐ b. At six weeks of age
- ☐ c. Within 12 hour of birth
- ☐ d. After one month of age

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Question 94

Not yet answered

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A two-hour-old male neonate was brought to a NICU because he had fast breathing. He was delivered at 38 weeks via cesarean delivery with birth weight of 3500 gm and Apgar score of 8 and 10 in the first and fifth minutes respectively. Examination revealed a respiratory rate of 65/min in otherwise vigorous baby. What is the most likely diagnosis for this baby?

- ☐ a. Congenital pneumonia
- ☐ b. Pulmonary hypoplasia
- ☐ c. Transient tachypnea of the newborn
- ☐ d. Meconium aspiration syndrome

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Question 96

Not yet answered

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A two-year-old female child presented to a hospital with a complaint offensive nasal discharge of three days duration. She also has a history of low-grade fever, cough and bad breath. Physical examination revealed bilateral erythematous and swollen nasal mucosa with purulent discharge. What is the most likely diagnosis in this child?

- ☐ a. Sinusitis
- ☐ b. Foreign body
- ☐ c. Lymphadenitis
- ☐ d. Periodontitis

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Question 98

Not yet answered

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 [Flag question](#)

A woman presented to a gynecologic referral clinic five months after she had got treatment for molar pregnancy. She attended her follow up without missing any schedule and her progress was rewarding. hCG is negative since her sixth visit and she does not have any complaint. She is taking combined oral contraceptive pills. What is the most appropriate next step in the management of this woman?

- ☐ a. Determining serum hCG
- ☐ b. Discontinuing contraception
- ☐ c. Prophylactic chemotherapy
- ☐ d. Order CXR

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Question 95

Not yet answered

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 [Flag question](#)

A two-month-old male infant presented with fast breathing and feeding interruption of two weeks duration. On examination, his pulse rate is 160/min, respirations are 60/min, temperature is 37.3 degree centigrade, oxygen saturation is 95percent, he has sub-costal and inter-costal retractions and total liver span is 9cm. What is the most likely diagnosis of this child?

- ☐ a. Pneumonia
- ☐ b. Perinatal tuberculosis
- ☐ c. Large VSD
- ☐ d. Severe anemia

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Question 99

Not yet answered

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 [Flag question](#)

An 18-year-old female patient came to a surgical OPD with a complaint of breast lump of three weeks duration. She has no pain and family history of similar illness. On evaluation, her vital signs are in the normal range and there is a freely mobile non-tender firm 2 x 2cm mass at right outer upper quadrant. What is the most appropriate next step in the patient's care?

- ☐ a. Send her for ultrasound and FNAC
- ☐ b. Reassure her and appoint after three months for evaluation
- ☐ c. Tell her it is fibroadenoma and send her home with analgesia
- ☐ d. Send her for mammography

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Question 97

Not yet answered

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 [Flag question](#)

A two-year-old male toddler was presented to an EOPD with abnormal body movement which lasts ten minutes for the first time. On evaluation, the child started to convulse again. His RBS was 112 mg/dl. What is the most appropriate initial management?

- ☐ a. Phenobarbital 20 mg/kg PO
- ☐ b. Phosphenytoin 20 mg/kg IV
- ☐ c. Diazepam rectally 0.1ml/kg
- ☐ d. Dextrose 10percent 5ml/kg bolus

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